

Hearing Loss or Change - Female (Adult)

Algorithm 1084 | Adult (18 years and older) | Gender at birth: Female

Definition

- Hearing loss/change assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Adult patient.
- Female.

Clinical background

- Hearing loss may be gradual or sudden, temporary or permanent, and can have treatable causes such as infection or earwax.
- Sudden hearing loss or deterioration over days or weeks requires urgent assessment.
- In children, hearing loss can affect speech development and school progress; caregiver observations are clinically important.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant, when did the hearing change start, was it sudden or gradual and one- or two-sided, and was there head injury, severe noise, foreign body, medicine exposure, neurological symptom, or safeguarding concern?
2. [YES_NO] One ear or both?
3. [YES_NO] Any ear pain, discharge, or dizziness?

Triage and redirect questions

1. Redirect → Earache

Is ear pain the main concern without new hearing loss or hearing change?

Primary ear pain without hearing change belongs in the earache pathway.

1. Disposition level 100

Does this sound like a life-threatening emergency to the triager (e.g. associated with a head injury or stroke symptoms)?

Universal emergency rule-out added ahead of the hearing loss guidance itself - hearing loss is rarely an emergency on its own, but must not mask trauma or a neurological event.

1. Disposition level 70

Did the hearing loss come on suddenly in one or both ears, has it worsened over just the last few days or weeks, or does it come with earache or discharge?

NHS.UK guidance recommends an urgent primary-care review or urgent clinical review call for sudden hearing loss or hearing loss with other symptoms - it may need to be treated quickly.

1. Disposition level 50

Has the hearing been getting gradually worse over time, or did it not improve after treatment for an ear infection or earwax?

NHS.UK guidance recommends a routine primary-care review for gradual hearing changes or hearing that hasn't improved after prior treatment.

Advice

Qatar emergency hearing-loss response

- Call Qatar emergency services on 999 and do not drive when hearing change follows major head trauma or accompanies stroke signs, severe vertigo with inability to stand, seizure, reduced consciousness, severe headache/neck stiffness, or fluid/blood from the ear after injury. Keep the person still; do not insert drops, tools, or packing into the ear and do not remove an embedded object. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: neurological change or reduced consciousness; severe vertigo or inability to stand; blood or clear fluid after head injury.

Urgent hearing review

- Arrange an urgent primary-care review or urgent clinical review assessment - sudden hearing changes may need quick treatment.
- Call Back If: hearing worsens further; dizziness develops.

Routine hearing follow-up

- Book a routine primary-care review; a free hearing test may also be available at some pharmacies and opticians.
- Call Back If: sudden worsening; new ear pain or discharge.

First aid

- Call Qatar emergency services on 999 and do not drive when hearing change follows major head trauma or accompanies stroke signs, severe vertigo with inability to stand, seizure, reduced consciousness, severe headache/neck stiffness, or fluid/blood from the ear after injury. Keep the person still; do not insert drops, tools, or packing into the ear and do not remove an embedded object. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Hearing loss", <https://www.nhs.uk/conditions/hearing-loss/> (page last reviewed 30 May 2025)

Provenance

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Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local disposition mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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